Lecture 18: What Happens When Things Go Wrong: Mental Illness, Part I

Introduction to Psychology · Yale University, Spring 2007 · Guest lecture by Professor Susan Nolen-Hoeksema

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Before you start

In this guest lecture, Professor Susan Nolen-Hoeksema explains how clinical psychology decides what counts as a mental disorder. She then uses mood disorders (depression and bipolar disorder) as a case example: how they are diagnosed, how common they are, which theories explain them, and how they are treated.

Learning goals

After this lecture you should be able to:

- Explain why diagnosis is a subjective judgment, and name three things that influence it.
- Describe the "three Ds" and the DSM criteria for major depression and for a manic episode.
- Compare unipolar depression and bipolar disorder.
- Give an example of evidence for the biological and cognitive theories of depression.
- Compare drug treatment and cognitive behavioral therapy, using the study with 240 patients.

Key ideas

1. Introduction to Dr Susan Nolen-Hoeksema

Professor Paul Bloom introduces the final guest lecturer of the semester. Susan Nolen-Hoeksema is a Yale psychology professor known for her research on depression, especially sex differences in depression. She is also an award-winning teacher, a textbook author and a popular writer.

2. Behavioral Criteria for Assessing Mental Disorders

- The first question of clinical psychology is "What is abnormality?" There is no biological test, such as a blood test, for any known mental disorder.
- Instead, clinicians use **behavioral criteria**: the symptoms a person reports and observations of their behavior. The decision is a fairly subjective "judgment call".
- This judgment is influenced by **social norms** (a woman wearing a veil is typical in a Muslim culture but was often seen as unusual in a non-Muslim culture), by **characteristics of the person** such as gender (a man crying seems more unusual than a woman crying), and by **context** (paranoia, always looking for threat, is adaptive in downtown Baghdad, but not on a quiet farm in Central Connecticut).
- Clinicians often use the **three Ds**: **distress** (to the person or to others), **dysfunction** in daily life, and **deviance** (highly unusual behavior). Deviance is the most controversial, because it depends so

much on social norms.

- In the United States, diagnosis uses the **Diagnostic and Statistical Manual (DSM)**. Since 1980 it has tried to use observable criteria that different clinicians agree on.

3. Unipolar Disorders

- **Unipolar** disorders involve depression only; **bipolar** disorders involve cycles between depression and mania (an abnormally high mood).
- For **major depression**, the DSM requires sadness or **anhedonia** (loss of interest or pleasure in usual activities), plus at least four other symptoms, such as appetite change, sleep problems, **psychomotor retardation** (slowed movement and speech), tiredness, worthlessness or guilt, poor concentration, and suicidal thoughts.
- Symptoms must last at least two weeks. Without treatment, an episode lasts on average at least six months.
- In a video, a woman named Tara describes her episodes. Depression is a **continuum** from everyday sadness to severe illness, with no clear dividing line.
- Many depressed people are the "**walking wounded**": they force themselves through the day and act normal, partly because they feel ashamed to seek help.

4. Bipolar Disorders

- A **manic episode** needs an abnormally elevated, expansive or irritable mood for at least one week, plus three or more symptoms, such as **grandiosity** (feeling like the smartest person on earth), less need for sleep, fast "pressured" talk, a **flight of ideas** (racing thoughts), and "pleasurable but dangerous activities" such as gambling.
- Mania is also a continuum. **Hypomania** is a milder form with the same symptoms; in a video, Bernie, a man with bipolar disorder, described the trouble his mania caused. In severe mania, false beliefs are grandiose: for example, believing one is the Messiah.
- A book by Kay Jamison argues that some famous people, such as Winston Churchill, had mild bipolar disorder and channeled it through great talent. But mostly, mania causes serious trouble, such as bankruptcy or arrest.
- Bipolar disorder affects about 1% of people, equally women and men. Depression is far more common: about 22% of women and about 13% of men will have an episode of serious depression in their lives.

5. Statistics for Depression

- In a nationwide study of people aged fifteen to fifty-five, the 15-24 age group had the highest rate of major depression. Rates in the elderly are quite low.
- Possible reasons: people get wiser with age; younger generations are more prone to depression; or people with lifelong depression die earlier. Nobody knows yet which is true.
- Before about age thirteen, boys and girls have similar levels of depression. From puberty, girls' rates rise, reaching almost two to one by age eighteen to twenty. Explanations include hormones and differences in stress and abuse.

6. Biological, Cognitive and Interpersonal Theories and Treatments

- **Genes:** If your identical twin has bipolar disorder, your chance of having it is over 60%; for a fraternal (non-identical) twin, about 12%. Avshalom Caspi and colleagues found that people with one or two "short alleles" (versions) of the serotonin transporter gene who were maltreated as children were more likely to become depressed. Without major stress, the gene made no difference: genes plus stress are a major risk factor.
- **Brain:** Brain chemicals called **neurotransmitters** (serotonin, norepinephrine and dopamine) are involved. The newer idea is that their **receptors** (the parts of neurons that receive them) do not work efficiently.
- **Drugs:** Older drugs called tricyclics help about 60% of people but have many side effects. Newer drugs called **SSRIs**, such as Prozac and Paxil, have fewer side effects. **Lithium** is the drug of choice for bipolar disorder.
- **Cognitive theory:** Aaron Beck says depressed people have a **negative cognitive triad**: a negative view of the self, the future and the world, fed by thinking errors such as "all-or-nothing thinking". A Temple University and University of Wisconsin study found that first-year students with a negative cognitive style were more likely to develop major depression later.
- **CBT:** Cognitive behavioral therapy helps people find and challenge negative thoughts. In a study of 240 patients, after sixteen weeks about 60% of both the Paxil group and the CBT group were no longer depressed. In the following year, almost 80% of those taken off the drug relapsed (became depressed again), compared with about 50% who stayed on Paxil and only about 35% after CBT.
- **Interpersonal therapy** links negative self-views to past relationships. It is less structured than CBT and has less research.

Key vocabulary

Term	Meaning in simple English	Example from the lecture
behavioral criteria	rules for diagnosis based on reported symptoms and observed behavior	There is no blood test, so clinicians use them
the three Ds	distress, dysfunction, deviance: signs that a behavior is abnormal	Depression causes high distress and dysfunction
DSM	the Diagnostic and Statistical Manual, used in the US to diagnose disorders	It lists the symptoms needed for major depression
anhedonia	loss of interest or pleasure in usual activities	"It's like the emotion has been sucked out of their life"
psychomotor retardation	slowed movement, reactions and speech	Depressed people get into more accidents
mania	a period of very high, expansive or irritable mood	The woman at the hairdresser's could not stop talking
hypomania	a milder form of mania with the same symptoms	Explained during the video of Bernie

Term	Meaning in simple English	Example from the lecture
continuum	a range with no clear line between its parts	From everyday sadness to severe depression
negative cognitive triad	a negative view of the self, the future and the world	Aaron Beck's theory of depression
relapse	becoming ill again after getting better	Almost 80% relapsed after stopping the drug

Check your understanding

1. Why does Professor Nolen-Hoeksema call diagnosis a "subjective judgment call"?
2. Name the three Ds and give an example of one of them.
3. What does the DSM require for a diagnosis of major depression?
4. How is a manic episode different from simply having a good day?
5. Compare depression and bipolar disorder: how common is each, and in whom?
6. Give two possible reasons for low rates of depression in older people.
7. What did Caspi and colleagues find about genes and stress?
8. In the study with 240 patients, which group had the lowest relapse rate, and why?

Answer guide

1. There is no biological test, so clinicians match symptoms and behavior to criteria. Social norms, the person's gender and the context can influence this judgment.
2. Distress, dysfunction and deviance. Example: depressed people often cannot go to class or work (dysfunction).
3. Sadness or anhedonia, plus at least four other symptoms, present for at least two weeks.
4. Mania is an abnormally elevated or irritable mood lasting at least one week, with symptoms such as grandiosity and racing thoughts.
5. About 22% of women and 13% of men will have serious depression. Bipolar disorder affects about 1%, equally women and men.
6. People may get wiser with age, or people with lifelong depression may die earlier. Nobody knows yet which explanation is true.
7. Short alleles of the serotonin transporter gene raised the risk of depression only together with stress, such as maltreatment in childhood.
8. The CBT group (about 35%). CBT teaches new coping skills for dealing with new stressors.

English practice

Useful phrases from the lecture

- "Where do we draw the line between…?" - asks where one category ends.
- "Well, the reality is that we don't." - corrects a common belief.
- "It's pretty clear that…" - introduces a conclusion with strong evidence.
- "It may be that they're all true to some extent." - shows careful, uncertain thinking.
- "This kind of finding has been replicated over and over again." - says a result is reliable.

Speaking task (with your AI tutor)

- Explain the difference between unipolar and bipolar disorders in 4-6 sentences. Use: *anhedonia*, *mania*, *continuum*.
- Explain how cognitive behavioral therapy works in 4-6 sentences. Use: *negative cognitive triad*, *challenge*, *relapse*.

Writing task

- Write one paragraph (80-120 words): "The Caspi study suggests that genes do not determine depression." Explain what the study found and why this conclusion follows.

Study with your tutor

Things to say to your tutor in Live conversation:

- "Let's review Lecture 18. Ask me questions about the DSM criteria for depression and mania, one at a time."

- "Quiz me on the key vocabulary of Lecture 18."
- "I will explain the three Ds in my own words. Check my understanding and my English."

Readings from the original course

- Gray, Peter. Psychology (5th edition), chapter 16
- Nasar, Sylvia. "A Beautiful Mind: The Life of Mathematical Genius and Nobel Laureate John Nash." In The Norton Psychology Reader. Edited by Gary Marcus. New York: W. W. Norton & Company, 2006. pp. 343-348
- Jamison, Kay Redfield. "An Unquiet Mind: A Memoir of Moods and Madness." In The Norton Psychology Reader. Edited by Gary Marcus. New York: W. W. Norton & Company, 2006. pp. 349-357

My notes